

**Physical Therapy
PT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Med	S80.12XD	Contusion of left lower leg, subsequent encounter	5/8/2026
Tx	R26.2	Difficulty in walking, not elsewhere classified	5/9/2026
Tx	M62.81	Muscle weakness (generalized)	5/9/2026

Plan of Treatment

Treatment Approaches May Include

- Therapeutic exercises (97110)
- Neuromuscular reeducation (97112)
- Gait training therapy (97116)
- Manual therapy techniques (97140)
- Group therapeutic procedure (97150)
- Physical therapy evaluation: moderate complexity (97162)
- Therapeutic activities (97530)

Frequency: 5 time(s)/week

Duration: 8 week(s)

Intensity: Daily

Cert. Period: 5/13/2026 - 6/11/2026

Objective Progress / Short-Term Goals

STG #1.0 - New Goal

Patient will enhance balance abilities as evidenced by an increase in score to 13/20 on the Elderly Mobility Scale. (Target: 6/16/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Elderly Mobility Scale		8/20

STG #2.0 - New Goal

Patient will safely ambulate on level surfaces 750 feet using SPC with Supervision or Touching Assistance with accurate direction of movement, with adequate weight acceptance, with functional loading response and with normal cadence to facilitate increased participation in functional activity. (Target: 6/16/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Gait	I	Partial/moderate assistance

STG #3.0 - New Goal

Patient will improve ability to safely and efficiently transfer to and from a bed to a chair (or wheelchair) with Supervision or Touching Assistance with ability to right self to achieve/maintain balance. (Target: 6/23/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Chair/bed-to-chair transfer	I	Partial/moderate assistance

**Physical Therapy
PT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Objective Progress / Short-Term Goals

STG #4.0 - New Goal

Patient will improve ability to complete toilet / commode transfers with Supervision or Touching Assistance with ability to right self to achieve/maintain balance. (Target: 6/9/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Toilet transfer	I	Partial/moderate assistance

STG #5.0 - New Goal

Patient will improve ability to safely go up and down four steps with Partial/Moderate Assistance using handrails bilaterally with ability to right self to achieve/maintain balance. (Target: 6/9/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
4 steps	I	Dependent

Objective Progress / Long-Term Goals

LTG #1.0 - New Goal

Patient will enhance functional mobility as evidenced by an increase in score to 15/20 on the Elderly Mobility Scale. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Elderly Mobility Scale		8/20

LTG #2.0 - New Goal

Patient will safely ambulate on level surfaces 1000 feet using SPC with Setup or Clean-up Assistance with accurate direction of movement, with adequate toe clearance and with normal cadence to facilitate increased participation in functional activity. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Gait	I	Partial/moderate assistance

LTG #3.0 - New Goal

Patient will improve ability to safely and efficiently transfer to and from a bed to a chair (or wheelchair) with Independence with ability to right self to achieve/maintain balance. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Chair/bed-to-chair transfer	I	Partial/moderate assistance

LTG #4.0 - New Goal

Patient will improve ability to complete toilet / commode transfers with Independence with ability to right self to achieve/maintain balance. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Toilet transfer	I	Partial/moderate assistance

LTG #5.0 - New Goal

Patient will safely ascend/descend 4 stairs with Setup or Clean-up Assistance using handrails bilaterally so that patient can safely discharge to next level of care. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Number of Stairs		0 steps
Stairs		Dependent

**Physical Therapy
PT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Patient Goals: Return to previous level of function .

Potential for Achieving Rehab Goals: Patient demonstrates excellent rehab potential as evidenced by ability to follow multi-step directions.

Focus of Plan of Treatment = Compensation, Adaptation

Participation = Patient/Caregiver participated in establishing POT

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/13/2026 12:01:49 PM EDT
Date

I certify the need for these medically necessary services furnished under this plan of treatment while under my care from 5/13/2026 through 6/11/2026.

☐ Physician Signature Not Required

Physician Signature: _____
Phuong Trinh, MD NPI: 1720585979 Date: 5/13/2026 6:43 PM EDT

Initial Assessment / Current Level of Function & Underlying Impairments

Patient Referral and History

Current Referral	Reason for Referral / Current Illness: Patient is an 89 y/o Male with a Pmhx of DM , HTN , Anemia , Back Surgery , L Knee Surgery recent hospitalization due to recent GLF at home resulting to R Distal Clavicular Fx , L Leg Pain and Swelling dx: B LE Cellulitis presenting with Difficulty in Ambulation , Generalized weakness requiring increase level of assistance with ADL'S / Reduce quality of life. Referred By = Discharge/Transition Team
Oxygen	Is Oxygen needed? = No
Weight Bearing	LE Weight Bearing Status = Weight Bearing as Tolerated
Medical Hx	Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery
Medical Factors	Precautions: Fall Risk , L LE Cellulitis Contraindications = No contraindications present.
Medications	Medications Impacting Condition/Treatment: Per consult with medical staff, patient is currently on prescribed medications for HYDROcodone-Acetaminophen Oral Tablet 5-325 MG (Hydrocodone-Acetaminophen) with possible side effects including dizziness and nausea which may affect patient's ability to process / think through situations safely and logically.
Labs	Abnormal lab values that may impact Treatment? = No
Prior Therapy	Did Patient Receive Therapy Previously? = Yes Location = Hospital Date(s) of Service: 5/8/2026 Prior Treatment Outcome: Excellent secondary to patient is motivated to return to previous level of function .
Prior Living	Prior Living Environment = Patient lived at home with others. Prior Living Description: Patient resides alone in SSH with 2 steps to get to the porch with 2 HR . He reported that he was a household ambulator using the SPC , Mod I with all transfers , ADL'S prior to recent onset. Current Assistance / Support = None Needed
Prior Equipment	Equipment Prior to Onset: SPC , FWW

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

**Physical Therapy
PT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Initial Assessment / Current Level of Function & Underlying Impairments

Functional Mobility Assessment

W/C Mobility Type of wheelchair or scooter used? = Manual
Other Picking up object = Partial/moderate assistance
Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 7
Mobility Performance Raw Score = 38
W/C Mngmnt. Armrests = Setup or clean-up assistance

Musculoskeletal System Assessment

LE ROM RLE ROM = WFL; LLE ROM = WFL
RLE Strength RLE Strength = WFL
LLE Strength LLE Strength = Impaired; Hip = Impaired; Knee = Impaired; Ankle = Impaired
(L) Hip Strength Flexion = 3-/5; Extension = 3-/5
(L) Knee Strength Flexion = 3-/5; Extension = 3-/5
(L) Ankle Strength Dorsiflexion = 3-/5; Plantar Flexion = 3-/5
Contracture Functional Limitations Present due to Contracture = No

Other System/Condition Assessment

Balance Patient sits unsupported x 30 seconds with feet flat on floor and no back support? = Yes; Patient stands without UE support w/AD as needed x 10 seconds? = No
Cardiovascular CardioPulmonary System = Functional for age and condition
Pain Patient has pain that interferes/limits functional activity? = Yes; Patient has pain that interferes with sleep? = Yes; Pain Assessment Method = Patient verbalized pain level.; Is skilled therapy needed to address pain? = Nursing to address

Objective Tests and Measures

Target Rate Target Heart Rate Range: 60-80
Test / LE Strength 30 Second Sit to Stand Test = Patient is Unable, requires use of hands
Did Patient need test modification = No
Test/Sit Balance Sitting Balance Scale = 37/44
Test / Balance / Gait Elderly Mobility Scale = 8/20

Assessment Summary

Communication Ability to Express Ideas and Wants = Understood; Ability to Understand Others = Understands; Follows 1-Step Directions = Independently
Cognition Decision Making Ability for Routine Activities = Independent
Reason for Therapy Clinical Impressions/Reason for Skilled Services: Based upon examination of patient's body regions, systems and structures, patient presents with balance deficits, decreased safety awareness, gross motor coordination deficits, postural alignment/control, safety awareness deficits and strength impairments and in consideration of history, personal factors, and functional limitations documented in this eval summary, patient requires skilled PT services to improve dynamic balance, facilitate motor control, minimize falls and increase independence with gait, in order to facilitate increased participation with functional daily activities. Due to the documented physical impairments and associated functional deficits, without skilled therapeutic intervention, the patient is at risk for: falls.
Complexities Barriers Likely to Impact Discharge to Next Level = None noted
Patient Characteristics that may Impact Treatment = None noted
Intervention Modes Can interventions be provided using treatment modes other than individual? = No

Exercise Prescription

Purpose Purpose of Exercise = Endurance, Strength
Method Method to Prescribe = 1 Rep Max (RM)
Resistance Resistance@ 1 RM = 2 pounds

**Physical Therapy
PT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Initial Assessment / Current Level of Function & Underlying Impairments

Exercise Prescription

Endurance 1 set / 12-25 reps @ 30% 1 RM, for Endurance: 1 set / 20 reps
Strength 1 set / 15 - 20 reps @ 40 - 60% 1 RM, for Strength: 1 set/ 20 reps

Physical Therapy
PT Evaluation & Plan of Treatment

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Physical Therapy

Identification Information

Patient: askew, James	DOB: 9/28/1936	Start of Care: 5/13/2026
Payer: Medicare Part A		
MRN: 6744		

**Occupational Therapy
OT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Tx	M62.81	Muscle weakness (generalized)	5/13/2026

Plan of Treatment

Treatment Approaches May Include

- Therapeutic exercises (97110)
- Neuromuscular reeducation (97112)
- Manual therapy techniques (97140)
- Group therapeutic procedure (97150)
- Occupational therapy evaluation: moderate complexity (97166)
- Therapeutic activities (97530)
- Self care management training (97535)

Frequency: 5 time(s)/week

Duration: 4 week(s)

Intensity: Daily

Cert. Period: 5/13/2026 - 6/11/2026

Objective Progress / Short-Term Goals

STG #1.0 - New Goal

Patient will improve ability to safely and efficiently perform LB dressing with Supervision or Touching Assistance in order to be able to return to prior level of living. (Target: 5/19/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Lower body dressing	I	Partial/moderate assistance

STG #2.0 - New Goal

Patient will improve ability to complete toilet / commode transfers with Supervision or Touching Assistance with ability to right self to achieve/maintain balance. (Target: 5/19/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Toilet transfer	I	Partial/moderate assistance

STG #3.0 - New Goal

Patient will improve ability to safely and efficiently maintain perineal hygiene, adjust clothes before/after voiding or having a bowel movement with Supervision or Touching Assistance in order to facilitate independence with toileting tasks. (Target: 5/19/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Toileting hygiene	I	Partial/moderate assistance

STG #4.0 - New Goal

Pt to engage in activity tolerance with pt completing 10 minutes of activity without rest break to promote increased activity tolerance. (Target: 5/19/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
	15+ minutes	2-3 minutes

**Occupational Therapy
OT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: Medicare Part A
MRN: 6744

Objective Progress / Long-Term Goals

LTG #1.0 - New Goal

Pt to demonstrate of F+ standing during ADLs and functional mobility to promote increased safety (Target: 6/11/2026)

PLOF (prior to onset)	Baseline (5/13/2026)
Fair+	fair-

LTG #2.0 - New Goal

Once standing, the patient will improve ability to safely ambulate 150 feet in a corridor or similar space with using two-wheeled walker with Setup or Clean-up Assistance. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
Walk 150 feet	I	Partial/moderate assistance

LTG #3.0 - New Goal

Patient will complete all ADL/self care tasks with Independence with ability to right self to achieve/maintain balance using AE as needed/trained. (Target: 6/11/2026)

	PLOF (prior to onset)	Baseline (5/13/2026)
LOA - GG	I	Independent

Patient Goals: regain strength and mobility to return home vs going to ALF

Potential for Achieving Rehab Goals: Patient demonstrates good rehab potential as evidenced by motivated to participate and strong family support.

Focus of Plan of Treatment = Restoration

Participation = Patient/Caregiver participated in establishing POT

Original Signature: _____ Electronically signed by Tiffany Simmons, OT 5/14/2026 11:55:01 AM EDT
Date

I certify the need for these medically necessary services furnished under this plan of treatment while under my care from 5/13/2026 through 6/11/2026.

☐ Physician Signature Not Required

Physician Signature: Electronically signed by Phuong Trinh, MD

Phuong Trinh, MD NPI: 1720585979 Date: 5/28/2026 12:14 PM EDT

Initial Assessment / Current Level of Function & Underlying Impairments

Patient Referral and History

Current Referral Reason for Referral / Current Illness: Pt is a 89 male s/p hospitalization with cc of LL pain after recent fall. pt found to have cellulitis and R clavicle from previous fall 1 month prior to presentation. Pt presents with impairments in balance, strength and activity tolerance impacting functional performance warranting skilled OT services.

Oxygen Is Oxygen needed? = No

Medical Hx Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery

Medical Factors Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Certification Period: 5/13/2026 - 6/11/2026
Occupational Therapy

Certification Period: 5/13/2026 - 6/11/2026
Occupational Therapy

**Occupational Therapy
OT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Occupational Therapy

Identification Information

Patient: askew, James	DOB: 9/28/1936	Start of Care: 5/13/2026
Payer: Medicare Part A		
MRN: 6744		

**Occupational Therapy
OT Evaluation & Plan of Treatment**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Certification Period: 5/13/2026 - 6/11/2026
Occupational Therapy

Identification Information

Patient: askew, James	DOB: 9/28/1936	Start of Care: 5/13/2026
Payer: Medicare Part A		
MRN: 6744		

Occupational Therapy OT Therapy Progress Report

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/19/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Tx	M62.81	Muscle weakness (generalized)	5/13/2026

Patient was seen for 5 day(s) during the 5/13/2026 - 5/19/2026 progress period.

Objective Progress / Short-Term Goals

STG #1.0 - Goal Met

Patient will improve ability to safely and efficiently perform LB dressing with Supervision or Touching Assistance in order to be able to return to prior level of living.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
Lower body dressing	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

STG #2.0 - Goal Met

Patient will improve ability to complete toilet / commode transfers with Supervision or Touching Assistance with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
Toilet transfer	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

STG #3.0 - Continue

Patient will improve ability to safely and efficiently maintain perineal hygiene, adjust clothes before/after voiding or having a bowel movement with Supervision or Touching Assistance in order to facilitate independence with toileting tasks.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
Toileting hygiene	I	Partial/moderate assistance	Partial/moderate assistance	Partial/moderate assistance

Comments:

STG #4.0 - Continue

Pt to engage in activity tolerance with pt completing 10 minutes of activity without rest break to promote increased activity tolerance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
	15+ minutes	2-3 minutes	2-3 minutes	5 min

Comments:

Objective Progress / Long-Term Goals

LTG #1.0 - Continue

Pt to demonstrate of F+ standing during ADLs and functional mobility to promote increased safety

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
	Fair+	fair-	fair-	fair -

Comments:

Occupational Therapy OT Therapy Progress Report

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/19/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Objective Progress / Long-Term Goals

LTG #2.0 - Continue

Once standing, the patient will improve ability to safely ambulate 150 feet in a corridor or similar space with using two-wheeled walker with Setup or Clean-up Assistance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
Walk 150 feet	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

LTG #3.0 - Continue

Patient will complete all ADL/self care tasks with Independence with ability to right self to achieve/maintain balance using AE as needed/trained.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/19/2026)
LOA - GG	I	Independent	Independent	Partial/moderate assistance

Comments:

Precautions / Prior Living / Discharge / Transition Plan

Medications Medications Impacting Condition/Treatment: Per consult with medical staff, patient is currently on prescribed medications for HYDROcodone-Acetaminophen Oral Tablet 5-325 MG (Hydrocodone-Acetaminophen) with possible side effects including dizziness and nausea which may affect patient's ability to process / think through situations safely and logically.

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Medical Hx Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery

Prior Living Prior Living Environment = Patient lived at home with others.

Prior Living Description: Patient resides alone in SSH with 2 steps to get to the porch with 2 HR . He reported that he was a household ambulator using the SPC , Mod I with all transfers , ADL'S prior to recent onset. Son assisting with heavy duty IADLs.

Prior Assistance Prior Cognitive Assistance = No supervision required

Prior Equipment Equipment Prior to Onset: SPC , FWW, grab bars

Discharge Plan Transition/Discharge Plan = Patient to live at home w/support/(A) from others.

Assistance/Support to be Provided = Community Assistance

Barriers Likely to Impact Discharge to Next Level = None noted

Patient Characteristics that may Impact Treatment = None noted

Functional Skills Assessment

Eating Eating = Independent

Hygiene Oral hygiene = Independent
Toileting hygiene = Partial/moderate assistance

Transfers Toilet transfer = Supervision or touching assistance

Bathing Shower/bathe self = Supervision or touching assistance

Dressing Upper body dressing = Supervision or touching assistance
Lower body dressing = Supervision or touching assistance
Putting on/taking off footwear = Supervision or touching assistance

Self-Care Score Self Care Function Score (score 0 - 12; 12 being the highest function) = 10
Self Care Performance Raw Score = 31

**Occupational Therapy
OT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/19/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Functional Skills Assessment

Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 1
Mobility Performance Raw Score = 4

Functional Skills Assessment - Activities of Daily Living (ADLs) & Instrumental ADLs

Hygiene Personal hygiene = Independent

Functional Skills Assessment - Functional Mobility

Bathing Tub/Shower transfer = Supervision or touching assistance

Assessment and Summary of Skilled Services

Interventions Skilled Interventions Provided: Direct, hands-on care with patient this reporting period focused on the following skilled interventions: ADLs, strength, endurance, balance, coordination, safety awareness and activity tolerance.

Patient Progress Progress & Response to Treatment: Patient is progressing with current treatment interventions and Plan Of Treatment.

Balance Patient sits unsupported x 30 seconds with feet flat on floor and no back support? = Yes; Patient stands without UE support w/AD as needed x 10 seconds? = No

Test/Sit Balance Sitting Balance Scale = NT

Purpose Purpose of Exercise = Strength

Method Method to Prescribe = Patient's Perceived Resistance

Resistance Resistance for Strength at Somewhat Hard to Hard Level = 2 pounds

Strength Muscle Groups@ 1 set, 15 - 20 reps, for Strength: all muscle groups of BUE for 2 sets of 10 reps

Supervision OT/Assistant Supervision: Therapist participated in direct patient care during this reporting period and Skilled services provided by therapist this reporting period, as well as with assistant.

Communication Team Communication/Collaboration: Treatment results communicated to Interdisciplinary Team and Functional skills reviewed with team members.

Justification for Continued Skilled Services

Impairments Remaining Impairments: strength impairments and balance deficits.

Continued Skill Reason for Skilled Services: Continued OT services are necessary to increase functional activity tolerance, increase independence with ADLs and increase safety awareness in order to be able to return to prior level of living and return to prior level of skill performance . Due to the documented physical impairments and associated functional deficits, without skilled therapeutic intervention, the patient is at risk for: falls.

Intervention Modes Can interventions be provided using treatment modes other than individual? = Yes; Will multiple modes provide a value-added benefit? = Yes; Will multiple modes enhance the therapeutic experience? = Yes; Is patient agreeable to modes other than individual? = Yes; Modes recommended = Group, Concurrent; Maximum number of patients for any group = 6; Justification for group recommendation: Patient will benefit from group therapy as an adjunct to individual therapy because group therapy facilitates the assessment and carryover of skills learned in individual therapy.; Justification for concurrent recommendation: Patient will benefit from concurrent therapy as an adjunct to individual therapy because concurrent therapy provides a therapeutic setting that facilitates generalization and carryover of skills learned in individual therapy.

Plan of Tx Focus Focus of Plan of Treatment = Restoration

Original Signature: _____ Electronically signed by Nicole Bardsley, COTA 5/21/2026 09:48:11 AM EDT
Date

Cosignature: _____ Electronically co-signed by Tiffany Simmons, OT 5/22/2026 12:47:14 PM EDT
Date

**Occupational Therapy
OT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/19/2026
Occupational Therapy

Identification Information

Patient: askew, James	DOB: 9/28/1936	Start of Care: 5/13/2026
Payer: UHC Navi (MGA)		
MRN: 6744		

**Physical Therapy
PT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/20/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Med	S80.12XD	Contusion of left lower leg, subsequent encounter	5/8/2026
Tx	R26.2	Difficulty in walking, not elsewhere classified	5/9/2026
Tx	M62.81	Muscle weakness (generalized)	5/9/2026

Patient was seen for 8 day(s) during the 5/13/2026 - 5/20/2026 progress period.

Objective Progress / Short-Term Goals

STG #1.0 - Continue

Patient will enhance balance abilities as evidenced by an increase in score to 13/20 on the Elderly Mobility Scale.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Elderly Mobility Scale		8/20	8/20	12/20

Comments:

STG #2.0 - Continue

Patient will safely ambulate on level surfaces 750 feet using SPC with Supervision or Touching Assistance with accurate direction of movement, with adequate weight acceptance, with functional loading response and with normal cadence to facilitate increased participation in functional activity.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Gait	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments: sup ~250' w/ RW

STG #3.0 - Continue

Patient will improve ability to safely and efficiently transfer to and from a bed to a chair (or wheelchair) with Supervision or Touching Assistance with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Chair/bed-to-chair transfer	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments: CGA for safety

STG #4.0 - Continue

Patient will improve ability to complete toilet / commode transfers with Supervision or Touching Assistance with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Toilet transfer	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments: CGA

STG #5.0 - Continue

Patient will improve ability to safely go up and down four steps with Partial/Moderate Assistance using handrails bilaterally with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
4 steps	I	Dependent	Dependent	Partial/moderate assistance

Comments: min w/ bil HR use

**Physical Therapy
PT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/20/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Objective Progress / Long-Term Goals

LTG #1.0 - Continue

Patient will enhance functional mobility as evidenced by an increase in score to 15/20 on the Elderly Mobility Scale.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Elderly Mobility Scale		8/20	8/20	12/20

Comments:

LTG #2.0 - Continue

Patient will safely ambulate on level surfaces 1000 feet using SPC with Setup or Clean-up Assistance with accurate direction of movement, with adequate toe clearance and with normal cadence to facilitate increased participation in functional activity.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Gait	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

LTG #3.0 - Continue

Patient will improve ability to safely and efficiently transfer to and from a bed to a chair (or wheelchair) with Independence with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Chair/bed-to-chair transfer	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

LTG #4.0 - Continue

Patient will improve ability to complete toilet / commode transfers with Independence with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Toilet transfer	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

LTG #5.0 - Continue

Patient will safely ascend/descend 4 stairs with Setup or Clean-up Assistance using handrails bilaterally so that patient can safely discharge to next level of care.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/13/2026)	Current (5/20/2026)
Number of Stairs		0 steps	0 steps	4 steps
Stairs		Dependent	Dependent	Partial/moderate assistance

Comments:

Precautions / Prior Living / Discharge / Transition Plan

Medications Medications Impacting Condition/Treatment: Per consult with medical staff, patient is currently on prescribed medications for HYDROcodone-Acetaminophen Oral Tablet 5-325 MG (Hydrocodone-Acetaminophen) with possible side effects including dizziness and nausea which may affect patient's ability to process / think through situations safely and logically.

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Weight Bearing LE Weight Bearing Status = Weight Bearing as Tolerated

**Physical Therapy
PT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/20/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Precautions / Prior Living / Discharge / Transition Plan

Medical Hx Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery

Prior Living Prior Living Environment = Patient lived at home with others.
Prior Living Description: Patient resides alone in SSH with 2 steps to get to the porch with 2 HR . He reported that he was a household ambulator using the SPC , Mod I with all transfers , ADL'S prior to recent onset.

Prior Assistance Prior Cognitive Assistance = No supervision required

Prior Equipment Equipment Prior to Onset: SPC , FWW

Discharge Plan Transition/Discharge Plan = Patient to live at home w/support/(A) from others.
Assistance/Support to be Provided = AM assistance/limited caregiver availability, PM assistance/limited caregiver availability
Barriers Likely to Impact Discharge to Next Level = None noted
Patient Characteristics that may Impact Treatment = None noted

Functional Skills Assessment

Bed Mobility Roll left and right = Independent
Sit to lying = Independent
Lying to sitting on side of bed = Independent

Transfers Sit to stand = Supervision or touching assistance
Chair/bed-to-chair transfer = Supervision or touching assistance
Toilet transfer = Supervision or touching assistance
Car transfer = Supervision or touching assistance

Ambulation Walk 10 feet = Supervision or touching assistance
Walk 50 feet with Two Turns = Supervision or touching assistance
Walk 150 feet = Supervision or touching assistance
Walking 10 feet on uneven surfaces = Supervision or touching assistance

Curbs/Stairs 1 step (curb) = Partial/moderate assistance
4 steps = Partial/moderate assistance
12 steps = Substantial/maximal assistance

W/C Mobility Resident uses a wheelchair and/or scooter? = No

Other Picking up object = Partial/moderate assistance

Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 10
Mobility Performance Raw Score = 61

Assessment and Summary of Skilled Services

Interventions Skilled Interventions Provided: Skilled treatment interventions included instructing and training patient in compensatory strategies, energy conservation techniques, environmental modifications, Functional Maintenance Program, positioning maneuvers, positioning/pressure relieving techniques, proper body mechanics, safe transfer techniques, safety precautions, self care/skin checks and use of assistive device(s) in order to achieve highest level of functional mobility.

Patient Progress Progress & Response to Treatment: Patient's functional abilities have potential to improve further as a result of skilled therapeutic interventions.

Balance Patient sits unsupported x 30 seconds with feet flat on floor and no back support? = Yes; Patient stands without UE support w/AD as needed x 10 seconds? = No

Target Heart Rate Target Heart Rate Range: 60-80

Test/Sit Balance Sitting Balance Scale = NT

Test / Balance / Gait Elderly Mobility Scale = 12/20

**Physical Therapy
PT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/20/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Assessment and Summary of Skilled Services

Purpose Purpose of Exercise = Endurance, Strength
Method Method to Prescribe = 1 Rep Max (RM)
Resistance Resistance@ 1 RM = 2 pounds
Endurance 1 set / 12-25 reps @ 30% 1 RM, for Endurance: 1 set / 20 reps
Strength 1 set / 15 - 20 reps @ 40 - 60% 1 RM, for Strength: 1 set/ 20 reps
Supervision PT/Assistant Supervision: Communication regarding the patient's objective progress and treatment progression has occurred with the treating clinician(s) this reporting period.
Communication Team Communication/Collaboration: Reviewed patient's plan of treatment and treatment services with interdisciplinary team members.

Justification for Continued Skilled Services

Impairments Remaining Impairments: balance deficits, decreased dynamic balance, decreased functional capacity, decreased safety awareness, decreased static balance and safety awareness deficits.
Continued Skill Reason for Skilled Services: Continued PT services are necessary in order to facilitate discharge planning, facilitate independence with all functional mobility, improve dynamic balance, increase functional activity tolerance, increase independence with gait, increase LE ROM and strength and promote safety awareness.
Intervention Modes Can interventions be provided using treatment modes other than individual? = No
Plan of Tx Focus Focus of Plan of Treatment = Restoration, Compensation

Original Signature: _____ Electronically signed by Samuel Wogaman, PTA 5/20/2026 03:18:00 PM EDT
Date

Cosignature: _____ Electronically co-signed by Maurice Chua, PT 5/21/2026 08:16:50 AM EDT
Date

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Identification Information

Patient: askew, James
MRN: 6744

DOB: 9/28/1936

Date of Service: 5/25/2026

Completed Date: 5/25/2026

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97112 97112: Patient up at BSC when PT arrived . No new complaints . Agreeable with PT . Ambulated to the gym using the RW with SBA . He participated in Neuroreeducation , Postural Stability training consisting of; obstacle course with rolling walker, weight shift multi-directional, step forward, forward reach and balloon tap , in standing, supported, 2 repetitions reps, 1 sets without resistance and moderate / 50% verbal instruction given due to compromised balance, aerobic capacity and functional activity tolerance in order to facilitate motor control and facilitate dynamic balance.

97116 97116: Patient ambulated approximately 400 ' x2 with SBA using the RW withn emphasis on; Skilled interventions to address Gait Training focused on facilitation of toe clearance at swing limb advancement, correct sequencing and hand foot placement during gait with assistive device and facilitation of knee flexion during walking.nstruced patient in compensatory strategies, energy conservation techniques, fall recovery techniques, positioning maneuvers, proper body mechanics, safe transfer techniques, safety precautions and use of assistive device(s) specifically, in order to facilitate improved performance during adl and iadl tasks, facilitate improved performance during functional activities, increase safety that may result from impairments/condition, prevent decline from current level of skill performance, increase safety and decrease need for assistance and increase functional mobility skills with 75% return demonstration provided during session.

Response to Tx Response to Session Interventions: compliant with adaptations.

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/25/2026 05:02:14 PM EDT
Date

Date of Service: 5/22/2026

Completed Date: 5/22/2026

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97150 97150: Patient participated in energy conservation group, therapeutic exercise group, environmental safety awareness group and functional activities group ; goal of group was to successfully progress patient with individual treatment plan/goals by addressing patient's balance deficits, decreased safety awareness, gross motor coordination deficits and joint mobility/integrity deficits ; skilled service provision included ; 3 patients participated in the group.

Response to Tx Response to Session Interventions: compliant with adaptations.

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/22/2026 03:11:20 PM EDT
Date

Date of Service: 5/21/2026

Completed Date: 5/21/2026

Summary of Daily Skilled Services

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97112 97112: Patient ambulated to the gyy using the RW . No new complaints . Agreeable with PT . Participated in Postural Stability training , Neuroreeducation consisting of; obstacle course with rolling walker, step laterally, weight shift lateral and forward reach , in standing, supported, 2 repetitions reps, 1 sets without resistance given due to compromised balance, aerobic capacity, functional activity tolerance and postural support/control in order to facilitate motor control and facilitate dynamic balance.

97116 97116: Patient ambulated approximately 400' with SBA -CGA using the RW with emphasis on; Skilled interventions to address Gait Training focused on adjustment of center of mass over base of support and correct sequencing and hand foot placement during gait with assistive device.

Response to Tx Response to Session Interventions: compliant with adaptations.

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/21/2026 04:37:45 PM EDT
Date

Date of Service: 5/20/2026 **Completed Date: 5/20/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97110 97110: Pt propelled nustep w/ LE's x15min lvl 5 to inc strength, ROM, and endurance for improved functional task performance

97112 97112: Pt performed standing step and bounce pass w/ orange swiss ball, mod/max for balance, pt performed forward step and forward weight shifting during pass, to improve dynamic balance for improved functional task performance

97116 97116: Pt amb ~250' w/ RW, PV, vc's for upright posture, to improve functional mobility+independence

97530 97530: Progress note completed in collaboration w/ patient and interdisciplinary staff in order to review PT POC, assess current functional status, and align goals to optimize rehabilitation potential in preparation for a safe discharge. Pt continues to show progress toward goals set in POC

Co-Treatment Co-Treatment occurred with = Occupational Therapy

Co-Treatment Details: Actively performed treatment w/ OT to achieve desired interventions targeting present deficits

Response to Tx Response to Session Interventions: Pt agreeable to therapy. actively participates with skilled interventions.

Complexities Barriers impacting treatment? = No

Supervision Supervising Therapist: Maurice Chua

Original Signature: _____ Electronically signed by Samuel Wogaman, PTA 5/20/2026 04:30:29 PM EDT
Date

Cosignature: _____ Electronically co-signed by Maurice Chua, PT 5/21/2026 08:16:33 AM EDT
Date

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Date of Service: 5/19/2026 **Completed Date: 5/19/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97110 97110: Pt propelled nustep w/ LE's x20min lvl 5 to inc strength, ROM, and endurance for improved functional task performance

97112 97112: Pt performed standing balloon tap using bil UE's, min a for balance, to improve gross motor movement, coordination, and standing dynamic balance on a stable surface

97116 97116: Pt amb ~250' x2 w/ RW, SPV, vc's for upright posture, to improve functional mobility+independence

Co-Treatment Co-Treatment occurred with = Occupational Therapy
Co-Treatment Details: Actively performed treatment w/ OT to achieve desired interventions targeting present deficits

Response to Tx Response to Session Interventions: Pt agreeable to therapy. actively participates with skilled interventions.

Complexities Barriers impacting treatment? = No

Supervision Supervising Therapist: Maurice Chua

Original Signature: _____ Electronically signed by Samuel Wogaman, PTA 5/19/2026 04:13:12 PM EDT
Date

Cosignature: _____ Electronically co-signed by Maurice Chua, PT 5/20/2026 08:03:28 AM EDT
Date

Revision Signature: _____ Electronically signed by Samuel Wogaman, PTA 5/29/2026 09:33:14 AM EDT
Date

[James askew] : Treatment Encounter Note(s)
Discipline: PT

Changes For: 5/29/2026 9:33:14 AM

Assessment

Changes	Modified By
Assessment added : Co-Treatment Details	Wogaman, Samuel
Assessment added : Co-Treatment occurred with	Wogaman, Samuel

Date of Service: 5/18/2026 **Completed Date: 5/18/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

97110 97110: Patient supine in bed when PT arrived . No new complaints . Agreeable with PT . Ambulated to the gym using the RW with CGA . He participated in Therapeutic Exercises , Postural Stability Training , Safety Awareness Training consisting of; Bilateral lower extremity therapeutic exercise focused on isometric exercise, progressive resistive exercise, terminal knee extension, hamstring isometrics and hamstring stretch , seated, 3 repetitions reps, 1 set(s), without resistance and moderate / 50% verbal instruction required due to compromised balance, aerobic capacity and functional activity tolerance to enhance muscle strength and enhance muscle performance in order to enhance safety and control during dynamic movement.

97112 97112: obstacle course with rolling walker, step backward and balloon toss , in standing, supported, 3 repetitions reps, 1 sets without resistance given due to compromised balance, aerobic capacity, functional activity tolerance and postural support/control in order to facilitate motor control and facilitate dynamic balance.

Response to Tx Response to Session Interventions: compliant with adaptations.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/19/2026 08:05:12 AM EDT
Date

Date of Service: 5/17/2026	Completed Date: 5/17/2026
Summary of Daily Skilled Services	

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97110 97110: Patient sitting up in chair, agreeable to participate with therapy.

Patient participated in therapeutic exercise, addressing patient's gross motor coordination deficits, muscle disuse/atrophy, postural alignment/control and strength impairments ; there ex included: LE, UE, and core strengthening, reciprocal marching while seated, motor sequencing, and body awareness (proprioception) for 2 sets of 10 with each extremity. (2#DB on UE extremity)

97116 97116: Patient ambulated ~250ft with RW, CGA; Skilled interventions to address Gait Training focused on base of support, adjustment of center of mass over base of support and self correction during task performance.

Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Complexities Barriers impacting treatment? = No

Supervision Supervising Therapist: M Chua

Original Signature: _____ Electronically signed by Shan Su, PTA 5/17/2026 04:19:43 PM EDT
Date

Cosignature: _____ Electronically co-signed by Maurice Chua, PT 5/18/2026 07:23:46 AM EDT
Date

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Date of Service: 5/16/2026 **Completed Date: 5/16/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97110 97110: 15 mins on the scit at lev adde resistance, 2x15 reps of resisted LE therex with a 2.5# AW, LAQ, hip flexion, hip abd and add with ball squeezes, resisted trunk rotations in order to improve joint mobility, muscles strength and stamina with occasional rest breaks in between tasks to recover from fatigue.

97116 97116: 200ft with SPC at min assist to maintain stability and all prevention. Skilled interventions to address gait training focused on facilitation of knee flexion during walking and facilitation of toe clearance at swing limb advancement.

Response to Tx Response to Session Interventions: Pt actively participates with skilled interventions and compliant with trained techniques.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Enyinnaya Nwamuo; PTA 5/16/2026 06:09:09 PM EDT
Date

Cosignature: _____ Electronically co-signed by Maurice Chua, PT 5/18/2026 07:23:46 AM EDT
Date

Date of Service: 5/15/2026 **Completed Date: 5/15/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97150 97150: Patient participated in functional activities group, functional mobility group and therapeutic exercise group ; goal of group was to successfully progress patient with individual treatment plan/goals by addressing patient's balance deficits, decreased safety awareness, gross motor coordination deficits and postural alignment/control ; skilled service provision included ; 2 patients participated in the group.

Response to Tx Response to Session Interventions: compliant with adaptations.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/15/2026 03:09:45 PM EDT
Date

Date of Service: 5/14/2026 **Completed Date: 5/15/2026**

Summary of Daily Skilled Services

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

97110 97110: Patient supine in bed when PT arrived . No new complaints . Agreeable with PT . He participated in Safety Awareness Training , Therapeutic Exercises , functional activity tolerance training consisting of STS 3x20 with CGA , Nustep Elliptical training x 20 minutes on level 3 with emphasis on energy conservation training ; Bilateral lower extremity therapeutic exercise focused on glut sets, closed chain exercise, progressive resistive exercise and terminal knee extension , seated, 2 repetitions reps, 1 set(s), without resistance and moderate / 50% verbal instruction required due to compromised balance, aerobic capacity, functional activity tolerance and postural support/control to enhance postural support/control and enhance muscle strength in order to facilitate ability to come to stance.

97116 97116: Patient ambulated approximately 200' with Min A using the RW with emphasis on; Skilled interventions to address Gait Training focused on facilitation of toe clearance at swing limb advancement, facilitation of symmetrical stance and facilitation of swing through.

Response to Tx Response to Session Interventions: compliant with adaptations.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/15/2026 08:10:28 AM EDT
Date

Date of Service: 5/13/2026	Completed Date: 5/13/2026
-----------------------------------	----------------------------------

Summary of Daily Skilled Services

Precautions Precautions: Fall Risk , L LE Cellulitis

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Target Heart Rate Range: 60-80

Evaluation 97162: Refer to the documented Plan of Treatment for patient history, comorbidities and/or personal factors that impact the plan, in addition to the examination findings and clinical presentation.

97530 97530: Skilled interventions focused on dynamic balance activities during sitting, facilitation of postural control, initiation cues to facilitate skill performance and transfer training to increase functional task performance 2 repetitions reps, 1 set(s), and moderate / 50% verbal instruction required due to compromised balance, aerobic capacity and functional activity tolerance.

Response to Tx Response to Session Interventions: compliant with adaptations.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Maurice Chua, PT 5/13/2026 03:39:13 PM EDT
Date

**Physical Therapy
PT Discharge Summary**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936
Payer: UHC Navi (MGA)
MRN: 6744

D/C Destination:

D/C Reason: Discharged per Physician or Case Manager

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Med	S80.12XD	Contusion of left lower leg, subsequent encounter	5/8/2026
Tx	R26.2	Difficulty in walking, not elsewhere classified	5/9/2026
Tx	M62.81	Muscle weakness (generalized)	5/9/2026

Patient was seen for 3 day(s) during the 5/21/2026 - 5/25/2026 progress period.

Objective Progress/Functional Comparison with Goals

Short-Term Goals

STG #1.0 - Discontinue on 05/25/2026

Patient will enhance balance abilities as evidenced by an increase in score to 13/20 on the Elderly Mobility Scale.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Elderly Mobility Scale		8/20	12/20	14/20

Comments:

STG #2.0 - Discontinue on 05/25/2026

Patient will safely ambulate on level surfaces 750 feet using SPC with Supervision or Touching Assistance with accurate direction of movement, with adequate weight acceptance, with functional loading response and with normal cadence to facilitate increased participation in functional activity.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Gait	I	Partial/moderate assistance	Supervision or touching assistance	Setup or clean-up assistance

Comments: Ambulatory approximately 800 ' with Sup using the RW with improved safety and postural awareness.

STG #3.0 - Discontinue on 05/25/2026

Patient will improve ability to safely and efficiently transfer to and from a bed to a chair (or wheelchair) with Supervision or Touching Assistance with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Chair/bed-to-chair transfer	I	Partial/moderate assistance	Supervision or touching assistance	Setup or clean-up assistance

Comments:

STG #4.0 - Discontinue on 05/25/2026

Patient will improve ability to complete toilet / commode transfers with Supervision or Touching Assistance with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Toilet transfer	I	Partial/moderate assistance	Supervision or touching assistance	Setup or clean-up assistance

Comments:

**Physical Therapy
PT Discharge Summary**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936
Payer: UHC Navi (MGA)
MRN: 6744

D/C Destination:

D/C Reason: Discharged per Physician or Case Manager

STG #5.0 - Discontinue on 05/25/2026

Patient will improve ability to safely go up and down four steps with Partial/Moderate Assistance using handrails bilaterally with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
4 steps	I	Dependent	Partial/moderate assistance	Supervision or touching assistance

Comments:

Long-Term Goals

LTG #1.0 - Discontinue on 05/25/2026

Patient will enhance functional mobility as evidenced by an increase in score to 15/20 on the Elderly Mobility Scale.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Elderly Mobility Scale		8/20	12/20	14/20

Comments:

LTG #2.0 - Discontinue on 05/25/2026

Patient will safely ambulate on level surfaces 1000 feet using SPC with Setup or Clean-up Assistance with accurate direction of movement, with adequate toe clearance and with normal cadence to facilitate increased participation in functional activity.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Gait	I	Partial/moderate assistance	Supervision or touching assistance	Setup or clean-up assistance

Comments:

LTG #3.0 - Discontinue on 05/25/2026

Patient will improve ability to safely and efficiently transfer to and from a bed to a chair (or wheelchair) with Independence with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Chair/bed-to-chair transfer	I	Partial/moderate assistance	Supervision or touching assistance	Setup or clean-up assistance

Comments:

LTG #4.0 - Discontinue on 05/25/2026

Patient will improve ability to complete toilet / commode transfers with Independence with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Toilet transfer	I	Partial/moderate assistance	Supervision or touching assistance	Setup or clean-up assistance

Comments:

**Physical Therapy
PT Discharge Summary**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Physical Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936
Payer: UHC Navi (MGA)
MRN: 6744

D/C Destination:

D/C Reason: Discharged per Physician or Case Manager

LTG #5.0 - Discontinue on 05/25/2026

Patient will safely ascend/descend 4 stairs with Setup or Clean-up Assistance using handrails bilaterally so that patient can safely discharge to next level of care.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/20/2026)	Discharge (5/25/2026)
Number of Stairs		0 steps	4 steps	8 steps
Stairs		Dependent	Partial/moderate assistance	Supervision or touching assistance

Comments:

Status / Prior Living / Discharge

Weight Bearing LE Weight Bearing Status = Weight Bearing as Tolerated
Medical Hx Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery
Prior Living Prior Living Environment = Patient lived at home with others.
Prior Living Description: Patient resides alone in SSH with 2 steps to get to the porch with 2 HR . He reported that he was a household ambulator using the SPC , Mod I with all transfers , ADL'S prior to recent onset.
Prior Assistance Prior Cognitive Assistance = No supervision required
Prior Equipment Equipment Prior to Onset: SPC , FWW
D/C Location Discharge Location = Patient discharged to ALF.
Assistance/Support to be Provided = AM assistance/limited caregiver availability, PM assistance/limited caregiver availability

Functional Skills Assessment

Bed Mobility Roll left and right = Independent
Sit to lying = Independent
Lying to sitting on side of bed = Independent
Transfers Sit to stand = Setup or clean-up assistance
Chair/bed-to-chair transfer = Setup or clean-up assistance
Toilet transfer = Setup or clean-up assistance
Car transfer = Not attempted due to environmental limitations
Assistive Device During Transfers = RW
Ambulation Walk 10 feet = Supervision or touching assistance
Walk 50 feet with Two Turns = Supervision or touching assistance
Walk 150 feet = Supervision or touching assistance
Walking 10 feet on uneven surfaces = Supervision or touching assistance
Gait Distance = 750 feet; Assistive Device = Two-wheeled Walker
Stairs/Curbs 1 step (curb) = Supervision or touching assistance
4 steps = Supervision or touching assistance
12 steps = Supervision or touching assistance
W/C Mobility Resident uses a wheelchair and/or scooter? = No
Other Picking up object = Supervision or touching assistance
Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 11
Mobility Performance Raw Score = 66

Physical Therapy PT Discharge Summary

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Physical Therapy

Identification Information

Patient:	askew, James	DOB:	9/28/1936
Payer:	UHC Navi (MGA)		
MRN:	6744		

D/C Destination:
D/C Reason: Discharged per Physician or Case Manager

Assessment and Summary of Skilled Services

Skilled Interventions	Skilled Interventions Provided: Skilled treatment interventions focused on education and training patient and caregivers in environmental modifications, energy conservation techniques, safe transfer techniques, safety precautions and compensatory strategies .
------------------------------	---

Patient Progress	Progress & Response to Treatment: Patient has reached maximum potential with skilled services.
-------------------------	--

Communication Team Communication/Collaboration: Functional skills reviewed with team members.

Test / LE Strength 30 Second Sit to Stand Test = 5
Did Patient need test modification = No

Test/Sit Balance Sitting Balance Scale = 42/44

Test / Balance / Gait Elderly Mobility Scale = 14/20

Discharge Recommendations and Status

D/C Recs	Discharge Recommendations: Home exercise program and Home health services.
-----------------	--

Target Heart Rate Target Heart Rate Range: 60-80

Restorative Programs	Restorative Program Established/Trained = Not Indicated at This Time
-----------------------------	--

Functional Maintenance	Functional Maintenance Program Established/Trained = Not Indicated at This Time
------------------------	---

Prognosis Prognosis to Maintain CLOF = Excellent

Original Signature:	Electronically signed by Maurice Chua, PT	5/27/2026 07:57:57 AM EDT
		Date

**Occupational Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Identification Information

Patient: askew, James
MRN: 6744

DOB: 9/28/1936

Date of Service: 5/25/2026

Completed Date: 5/25/2026

Summary of Daily Skilled Services

Precautions Precautions: Fall risk,
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97110 97110: Pt used NuStep X 20 min focusing on increasing dynamic sitting balance and coordination on level 5 resistance with minimal rest breaks showing good recovery following rest breaks.
97112 97112: Pt engaged in dynamic standing balance and functional ambulation activity focusing on increasing balance with bending when reaching for objects out of pt's reach and picking up objects off floor.
97535 97535: Pt demonstrated modl with toilet transfers.
Concurrent Tx Concurrent Treatment: Pt completed part of today's OT session with one additional pt in therapy gym at time of today's OT session.
Response to Tx Response to Session Interventions: Pt is progressing towards his OT goals and showed good response to today's OT session. Con't POC per supervising OTR.
Supervision Supervising Therapist: A. Joy

Original Signature: _____ Electronically signed by Leon King, COTA 5/25/2026 12:56:02 PM EDT
Date

Cosignature: _____ Electronically co-signed by Anita Joy, OT 5/27/2026 03:31:53 PM EDT
Date

Date of Service: 5/22/2026

Completed Date: 5/22/2026

Summary of Daily Skilled Services

Precautions Precautions: Fall risk,
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97530 97530: Skilled interventions focused on dynamic balance activities while standing and dynamic functional activities to increase strength, range of motion, flexibility in a progressive manner 5 repetitions reps, 2 set(s), and fading verbal instruction required due to compromised functional activity tolerance.
Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Original Signature: _____ Electronically signed by Christina Garcia, COTA 5/22/2026 08:53:08 PM EDT
Date

Cosignature: _____ Electronically co-signed by Anita Joy, OT 5/27/2026 03:31:53 PM EDT
Date

Date of Service: 5/21/2026

Completed Date: 5/21/2026

Summary of Daily Skilled Services

**Occupational Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Precautions Precautions: Fall risk,
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97112 97112: ball toss and weight shift anterior with unsupported sit to stands, 10 repetitions reps, 3 sets without resistance and moderate / 50% physical assistance and moderate / 50% verbal instruction given due to compromised balance and comprehension in order to provide corrective assistance for loss of balance and provide proprioceptive techniques to improve safety.
97530 97530: Progress note completed in collaboration with patient and interdisciplinary staff in order to review OT POC, assess current functional status and align goals to optimize rehabilitation potential in preparation for a safe discharge. Pt continues to demonstrate motivation and active engagement toward achieving established outcomes. Skilled interventions focused on strengthening activities to increase functional task performance and closed kinetic chain activity to increase functional skills for 15 mins with intermittent rest breaks.
Response to Tx Response to Session Interventions: compliant with skilled interventions and increased challenge with tasks.
Supervision Supervising Therapist: T Simons

Original Signature:	Electronically signed by Nicole Bardsley, COTA	5/21/2026 05:12:16 PM EDT
		Date

Cosignature:	Electronically co-signed by Tiffany Simmons, OT	5/22/2026 12:47:02 PM EDT
		Date

**Occupational Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Date of Service: 5/20/2026 **Completed Date: 5/20/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97110 97110: Trunk Exercise focused on abdominal curls seated, 10 repetitions reps, 2 set(s) and moderate / 50% tactile cues/facilitation and moderate / 50% physical assistance required due to compromised strength to enhance muscle strength in order to improve balance during ADL / IADL performance.

97112 97112: Facilitation of dynamic balance act while standing to toss ball using BUEs with 1 step FW with each toss > to increase strength, endurance, balance, crossing midline, overhead reaching, safety and activity tolerance. Pt demo moderate LOB posteriorly and required Mod-Max assistance to correct balance.

97535 97535: Pt completed oral hygiene and hair grooming while standing at sink with SBA due to safety awareness. No LOB noted and min VCs for safety awareness needed.

Co-Treatment Co-Treatment occurred with = Physical Therapy

Co-Treatment Details: Patient participated in therapy in co-treatment mode to successfully progress towards individual treatment plan/goals by addressing patient's strength impairments and balance deficits.

Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Supervision Supervising Therapist: T Simmons

Original Signature: _____ Electronically signed by Nicole Bardsley, COTA 5/20/2026 06:41:41 PM EDT
Date

Cosignature: _____ Electronically co-signed by Tiffany Simmons, OT 5/22/2026 12:47:02 PM EDT
Date

Revision Signature: _____ Electronically signed by Nicole Bardsley, COTA 5/29/2026 09:56:59 AM EDT
Date

[James askew] : Treatment Encounter Note(s)
Discipline: OT

Changes For: 5/29/2026 9:56:59 AM

Assessment

Changes	Modified By
Assessment added : Co-Treatment Details	Bardsley, Nicole
Assessment added : Co-Treatment occurred with	Bardsley, Nicole

Date of Service: 5/19/2026 **Completed Date: 5/19/2026**

Summary of Daily Skilled Services

**Occupational Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97112 97112: Patient participated in dynamic standing balance activity to tap balloon with fading MOD A for balance> to work on balance skills, crossing midline, overhead reaching, standing tolerance and activity tolerance to increase (I) with ADLs. Pt required min VCs to decrease wide BoS and to shift weight FW.

97530 97530: Skilled interventions focused on strengthening activities to increase functional task performance and gross motor coordination 10 repetitions reps, 2 set(s), and minimal / intermittent visual demonstration required due to compromised coordination.

Co-Treatment Co-Treatment occurred with = Physical Therapy

Co-Treatment Details: Patient participated in therapy in co-treatment mode to successfully progress towards individual treatment plan/goals by addressing patient's balance deficits.

Response to Tx Response to Session Interventions: actively participates with skilled interventions and compliant with skilled interventions.

Complexities Barriers impacting treatment? = No

Supervision Supervising Therapist: T Simmons

Original Signature: _____ Electronically signed by Nicole Bardsley, COTA 5/19/2026 03:39:11 PM EDT
Date

Cosignature: _____ Electronically co-signed by Anita Joy, OT 5/19/2026 03:54:16 PM EDT
Date

Date of Service: 5/18/2026 **Completed Date: 5/18/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97110 97110: Pt completed 20 mins of BUE AROM via UBE with moderate resistance to work on increasing strength, endurance, aerobic conditioning and act tol for ADL performances.

97530 97530: Facilitation of dynamic balance act while standing to toss ball using BUEs w/o UB support to increase strength, endurance, balance, crossing midline, overhead reaching, safety and activity tolerance. Pt demo moderate LOB posteriorly and required moderate assistance to correct balance.

Pt ambulated long distances using 2WW with intermittent CGA due to improper use of walker with turns; educated pt on proper technique with partial carryover due to impaired safety awareness.

Response to Tx Response to Session Interventions: actively participates with skilled interventions and compliant with skilled interventions.

Complexities Barriers impacting treatment? = No

**Occupational Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Supervision Supervising Therapist: T Simmons

Original Signature: _____ Electronically signed by Nicole Bardsley, COTA 5/18/2026 04:54:07 PM EDT
Date

Cosignature: _____ Electronically co-signed by Tiffany Simmons, OT 5/18/2026 06:30:11 PM EDT
Date

Date of Service: 5/15/2026 **Completed Date: 5/15/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97530 97530: Pt ambulated long distances from gym to room using 2WW with CGA-SBA and min VCs due to easily distracted.

97150 97150: Patient participated in therapeutic exercise group ; goal of group was to successfully progress patient with individual treatment plan/goals by addressing patient's strength impairments, decreased dynamic balance and decreased functional capacity ; skilled service provision included BUE ther ex in all planes as tolerated, core strengthening, dynamic sitting balance and activity tolerance ; 4 patients participated in the group.

Response to Tx Response to Session Interventions: actively participates with skilled interventions and compliant with skilled interventions.

Complexities Barriers impacting treatment? = No

Supervision Supervising Therapist: T Simmons

Original Signature: _____ Electronically signed by Nicole Bardsley, COTA 5/15/2026 05:01:14 PM EDT
Date

Cosignature: _____ Electronically co-signed by Tiffany Simmons, OT 5/15/2026 06:50:57 PM EDT
Date

Date of Service: 5/14/2026 **Completed Date: 5/15/2026**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97530 97530: Skilled interventions focused on strengthening activities to increase functional task performance 10 repetitions reps, 3 set(s), and mild / 25% verbal instruction required due to compromised technique. Pt engaged in activity tolerance training with pt completing 15 minutes with rest break after 8 minutes . Pt provided with VC for pacing to promote energy conservation

**Occupational Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF DALLAS

askew, James

Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Tiffany Simmons, OT 5/15/2026 09:06:23 PM EDT
Date

Date of Service: 5/13/2026	Completed Date: 5/15/2026
-----------------------------------	----------------------------------

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, R clavicle fx (awaiting orders for mobility/WB for RUE)

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

Evaluation 97166: Refer to the documented Plan of Treatment for occupational profile, medical and therapy history and/or comorbidities that impact the Plan, as well as assessment findings/performance deficits and modification/assistance needs.

97530 97530: Skilled interventions focused on transfer training to increase functional task performance 2 repetitions reps, 1 set(s), and mild / 25% physical assistance required due to compromised technique and safety awareness.

97535 97535: Skilled interventions to facilitate safety and independence with Toileting tasks focused on initiation cues to facilitate skill performance, instruction in toileting/clothing management techniques and safety training during functional mobility during toileting.

Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Complexities Barriers impacting treatment? = No

Original Signature: _____ Electronically signed by Tiffany Simmons, OT 5/15/2026 07:53:33 PM EDT
Date

Occupational Therapy OT Therapy Progress Report

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/20/2026 - 5/25/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Tx	M62.81	Muscle weakness (generalized)	5/13/2026

Patient was seen for 4 day(s) during the 5/20/2026 - 5/25/2026 progress period.

Objective Progress / Short-Term Goals

STG #3.0 - Goal Met

Patient will improve ability to safely and efficiently maintain perineal hygiene, adjust clothes before/after voiding or having a bowel movement with Supervision or Touching Assistance in order to facilitate independence with toileting tasks.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Current (5/25/2026)
Toileting hygiene	I	Partial/moderate assistance	Partial/moderate assistance	Independent

Comments:

STG #4.0 - Continue

Pt to engage in activity tolerance with pt completing 10 minutes of activity without rest break to promote increased activity tolerance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Current (5/25/2026)
	15+ minutes	2-3 minutes	5 min	5 min

Comments:

Objective Progress / Long-Term Goals

LTG #1.0 - Continue

Pt to demonstrate of F+ standing during ADLs and functional mobility to promote increased safety

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Current (5/25/2026)
	Fair+	fair-	fair -	fair

Comments:

LTG #2.0 - Continue

Once standing, the patient will improve ability to safely ambulate 150 feet in a corridor or similar space with using two-wheeled walker with Setup or Clean-up Assistance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Current (5/25/2026)
Walk 150 feet	I	Partial/moderate assistance	Supervision or touching assistance	Supervision or touching assistance

Comments:

LTG #3.0 - Continue

Patient will complete all ADL/self care tasks with Independence with ability to right self to achieve/maintain balance using AE as needed/trained.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Current (5/25/2026)
LOA - GG	I	Independent	Partial/moderate assistance	Partial/moderate assistance

Comments:

Dates of Service: 5/20/2026 - 5/25/2026
Occupational Therapy

Patient:	askew, James	DOB:	9/28/1936	Start of Care:	5/13/2026
Payer:	UHC Navi (MGA)				
MRN:	6744				

Medications	Medications Impacting Condition/Treatment: Per consult with medical staff, patient is currently on prescribed medications for HYDROcodone-Acetaminophen Oral Tablet 5-325 MG (Hydrocodone-Acetaminophen) with possible side effects including dizziness and nausea which may affect patient's ability to process / think through situations safely and logically.
Precautions	Precautions: Fall risk,
Contraindications	Contraindications = No contraindications present.
Medical Hx	Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery
Prior Living	Prior Living Environment = Patient lived at home with others. Prior Living Description: Patient resides alone in SSH with 2 steps to get to the porch with 2 HR . He reported that he was a household ambulator using the SPC , Mod I with all transfers , ADL'S prior to recent onset. Son assisting with heavy duty IADLs.
Prior Assistance	Prior Cognitive Assistance = No supervision required
Prior Equipment	Equipment Prior to Onset: SPC , FWW, grab bars
Discharge Plan	Transition/Discharge Plan = Patient to live at home w/support/(A) from others. Assistance/Support to be Provided = Community Assistance Barriers Likely to Impact Discharge to Next Level = None noted Patient Characteristics that may Impact Treatment = None noted

Eating	Eating = Independent
Hygiene	Oral hygiene = Independent Toileting hygiene = Independent
Transfers	Toilet transfer = Supervision or touching assistance
Bathing	Shower/bathe self = Supervision or touching assistance
Dressing	Upper body dressing = Independent Lower body dressing = Partial/moderate assistance Putting on/taking off footwear = Substantial/maximal assistance
Self-Care Score	Self Care Function Score (score 0 - 12; 12 being the highest function) = 12 Self Care Performance Raw Score = 33
Bed Mobility	Roll left and right = Independent Sit to lying = Independent Lying to sitting on side of bed = Independent
Transfers	Sit to stand = Independent Chair/bed-to-chair transfer = Independent Assistive Device During Transfers = RW
Mobility Score	Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 8 Mobility Performance Raw Score = 34

Hygiene Personal hygiene = Independent

Bathing	Tub/Shower transfer = Supervision or touching assistance
----------------	--

Interventions	Skilled Interventions Provided: U UE strengthening, transfers, balance, edurance, mobility and ADLs have all been addressed as part of skilled OT services provided in SNF
----------------------	--

**Occupational Therapy
OT Therapy Progress Report**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/20/2026 - 5/25/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936 **Start of Care:** 5/13/2026
Payer: UHC Navi (MGA)
MRN: 6744

Assessment and Summary of Skilled Services

Patient Progress Progress & Response to Treatment: Pt is progressing towards his OT goals and showing good response to OT intervention sessions

Balance Patient sits unsupported x 30 seconds with feet flat on floor and no back support? = Yes; Patient stands without UE support w/AD as needed x 10 seconds? = No

Target Heart Rate Target Heart Rate Range: WNL

Test/Sit Balance Sitting Balance Scale = NT

Purpose Purpose of Exercise = Strength

Method Method to Prescribe = Patient's Perceived Resistance

Resistance Resistance for Strength at Somewhat Hard to Hard Level = 2 pounds

Strength Muscle Groups@ 1 set, 15 - 20 reps, for Strength: all muscle groups of BUE for 2 sets of 10 reps

Supervision OT/Assistant Supervision: OTA is collaborating with supervising OTR regarding pt's progress, goals and POC

Communication Team Communication/Collaboration: OTA is collaborating supervising OTR, PT and PTA regarding pt's progress, goals and POC

Justification for Continued Skilled Services

Impairments Remaining Impairments: Pt is still showing decline in the areas of U UE strengthening, transfers, balance, endurance, mobility and ADLs compared to her PLOF

Continued Skill Reason for Skilled Services: Pt remains a good OT candidate to continue addressing U UE strengthening, transfers, balance, endurance, mobility and ADLs prior to d/c from SNF

Intervention Modes Can interventions be provided using treatment modes other than individual? = Yes; Will multiple modes provide a value-added benefit? = Yes; Will multiple modes enhance the therapeutic experience? = Yes; Is patient agreeable to modes other than individual? = Yes; Modes recommended = Group, Concurrent; Maximum number of patients for any group = 6; Justification for group recommendation: Patient will benefit from group therapy as an adjunct to individual therapy because group therapy facilitates the assessment and carryover of skills learned in individual therapy.; Justification for concurrent recommendation: Patient will benefit from concurrent therapy as an adjunct to individual therapy because concurrent therapy provides a therapeutic setting that facilitates generalization and carryover of skills learned in individual therapy.

Plan of Tx Focus Focus of Plan of Treatment = Restoration, Compensation

Original Signature: _____ Electronically signed by Leon King, COTA 5/25/2026 01:01:52 PM EDT
Date

Cosignature: _____ Electronically co-signed by Anita Joy, OT 5/27/2026 03:33:01 PM EDT
Date

**Occupational Therapy
OT Discharge Summary**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936
Payer: UHC Navi (MGA)
MRN: 6744

D/C Destination:
D/C Reason: Highest Practical Level Achieved

Progress Period: 5/25/2026 - 5/25/2026

Diagnoses

Type	Code	Description	Onset
Med	L03.116	Cellulitis of left lower limb	5/8/2026
Tx	M62.81	Muscle weakness (generalized)	5/13/2026

Objective Progress/Functional Comparison with Goals

Short-Term Goals

STG #1.0 - Met on 05/19/2026

Patient will improve ability to safely and efficiently perform LB dressing with Supervision or Touching Assistance in order to be able to return to prior level of living.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/19/2026)	Discharge (5/25/2026)
Lower body dressing	I	Partial/moderate assistance	Supervision or touching assistance	Met on 05/19/2026

Comments:

STG #2.0 - Met on 05/19/2026

Patient will improve ability to complete toilet / commode transfers with Supervision or Touching Assistance with ability to right self to achieve/maintain balance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/19/2026)	Discharge (5/25/2026)
Toilet transfer	I	Partial/moderate assistance	Supervision or touching assistance	Met on 05/19/2026

Comments:

STG #3.0 - Met on 05/25/2026

Patient will improve ability to safely and efficiently maintain perineal hygiene, adjust clothes before/after voiding or having a bowel movement with Supervision or Touching Assistance in order to facilitate independence with toileting tasks.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Discharge (5/25/2026)
Toileting hygiene	I	Partial/moderate assistance	Partial/moderate assistance	Supervision or touching assistance

Comments:

STG #4.0 - Met on 05/25/2026

Pt to engage in activity tolerance with pt completing 10 minutes of activity without rest break to promote increased activity tolerance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Discharge (5/25/2026)
	15+ minutes	2-3 minutes	5 min	10

Comments:

**Occupational Therapy
OT Discharge Summary**

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Occupational Therapy

Identification Information

Patient: askew, James **DOB:** 9/28/1936
Payer: UHC Navi (MGA)
MRN: 6744

D/C Destination:
D/C Reason: Highest Practical Level Achieved

Long-Term Goals

LTG #1.0 - Met on 05/25/2026

Pt to demonstrate of F+ standing during ADLs and functional mobility to promote increased safety

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Discharge (5/25/2026)
	Fair+	fair-	fair -	Fair +

Comments:

LTG #2.0 - Met on 05/25/2026

Once standing, the patient will improve ability to safely ambulate 150 feet in a corridor or similar space with using two-wheeled walker with Setup or Clean-up Assistance.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Discharge (5/25/2026)
Walk 150 feet	I	Partial/moderate assistance	Supervision or touching assistance	Independent

Comments:

LTG #3.0 - Discontinue on 05/25/2026

Patient will complete all ADL/self care tasks with Independence with ability to right self to achieve/maintain balance using AE as needed/trained.

	PLOF (prior to onset)	Baseline (5/13/2026)	Previous (5/25/2026)	Discharge (5/25/2026)
LOA - GG	I	Independent	Partial/moderate assistance	Setup or clean-up assistance

Comments:

Status / Prior Living / Discharge

Medical Hx Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery
Prior Living Prior Living Environment = Patient lived at home with others.
Prior Living Description: Patient resides alone in SSH with 2 steps to get to the porch with 2 HR . He reported that he was a household ambulator using the SPC , Mod I with all transfers , ADL'S prior to recent onset. Son assisting with heavy duty IADLs.
Prior Assistance Prior Cognitive Assistance = No supervision required
Prior Equipment Equipment Prior to Onset: SPC , FWW, grab bars
D/C Location Discharge Location = Patient discharged to ALF.
Assistance/Support to be Provided = Community Assistance

Functional Skills Assessment

Eating Eating = Independent
Hygiene Oral hygiene = Independent
Toileting hygiene = Supervision or touching assistance
Transfers Toilet transfer = Independent
Bathing Shower/bathe self = Supervision or touching assistance
Dressing Upper body dressing = Setup or clean-up assistance
Lower body dressing = Supervision or touching assistance
Putting on/taking off footwear = Supervision or touching assistance
Self-Care Score Self Care Function Score (score 0 - 12; 12 being the highest function) = 11

Occupational Therapy OT Discharge Summary

Provider: PH OPS OF DALLAS
NPI: 1790422822

Dates of Service: 5/13/2026 - 5/25/2026
Occupational Therapy

Identification Information

Patient:	askew, James	DOB:	9/28/1936
Payer:	UHC Navi (MGA)		
MRN:	6744		

D/C Destination:
D/C Reason: Highest Practical Level Achieved

Functional Skills Assessment

Self-Care Score	Self Care Performance Raw Score = 33
Mobility Score	Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 1 Mobility Performance Raw Score = 6

Functional Skills Assessment - Activities of Daily Living (ADLs) & Instrumental ADLs

Hygiene Personal hygiene = Independent

Functional Skills Assessment - Functional Mobility

Bathing Tub/Shower transfer = Setup or clean-up assistance

Assessment and Summary of Skilled Services

Skilled Interventions	Skilled Interventions Provided: Skilled treatment interventions focused on education and training patient and caregivers in proper body mechanics, safe transfer techniques, safety precautions and self care/skin checks in order to .
Patient Progress	Progress & Response to Treatment: Patient demonstrates little to no physical impairments as a result of skilled rehab and Patient has made consistent progress with skilled interventions.
Communication	Team Communication/Collaboration: Care planning conference held with interdisciplinary team this PR period - functional skills and discharge plans reviewed and Functional skills reviewed with team members.
Test/Sit Balance	Sitting Balance Scale = 44/44
Tests / Measures	Tests/Measures: Barthel Index 65/100

Discharge Recommendations and Status

D/C Recs	Discharge Recommendations: Assistive device for safe functional mobility.
Target Heart Rate	Target Heart Rate Range: WFL
Restorative Programs	Restorative Program Established/Trained = Not Indicated at This Time
Functional Maintenance	Functional Maintenance Program Established/Trained = Not Indicated at This Time
Prognosis	Prognosis to Maintain CLOF = Good with strong family support

Original Signature:	Electronically signed by Anita Joy, OT	5/28/2026 02:47:54 PM EDT
		Date